

HIGHLIGHTS OF PRESCRIBING INFORMATION: PARACETAMOL (ACETAMINOPHEN)

WARNING: LIVER TOXICITY (HEPATOTOXICITY)

Acetaminophen has been associated with cases of acute liver failure, at times resulting in liver transplant and death. Most of the cases of liver injury are associated with the use of acetaminophen at doses that exceed 4000 milligrams per day, and often involve more than one acetaminophen-containing product.

1 INDICATIONS AND USAGE

Paracetamol (Acetaminophen) is indicated for:

- Relief of mild to moderate pain including headache, toothache, muscle aches, backache, osteoarthritis, and menstrual cramps.
- Reduction of fever in adults and children.

2 DOSAGE AND ADMINISTRATION

- Adults and Adolescents (≥ 12 years): 325 mg to 650 mg every 4 to 6 hours or 1000 mg every 6 hours as needed.
- Maximum Daily Dose: Do not exceed 4000 mg (4 grams) in 24 hours from all sources in adults. In chronic alcohol users or patients with hepatic impairment, maximum daily dose should not exceed 2000 mg to 3000 mg.
- Pediatric Patients (< 12 years): 10 to 15 mg/kg per dose every 4 to 6 hours as needed (maximum 5 doses or 75 mg/kg per day).
- Do not use with any other drug containing acetaminophen/paracetamol.

4 CONTRAINDICATIONS

- Hypersensitivity to acetaminophen or any component of the formulation.
- Severe acute liver impairment or active severe hepatic disease.

5 WARNINGS AND PRECAUTIONS

- Hepatotoxicity: Do not exceed recommended maximum daily dose. Avoid use with other acetaminophen-containing medications. Chronic alcohol consumption (> 3 drinks per day) increases hepatotoxicity risk.
- Serious Skin Reactions: Rarely, paracetamol may cause serious skin reactions such as acute generalized exanthematous pustulosis (AGEP), Stevens-Johnson Syndrome (SJS), and toxic epidermal necrolysis (TEN). Discontinue immediately at first appearance of skin rash.

6 ADVERSE REACTIONS

Generally well tolerated at therapeutic doses.

- Rare adverse reactions: Allergic skin eruptions, urticaria, erythema, thrombocytopenia, leukopenia, neutropenia.
- Hepatic: Elevated liver enzymes (ALT, AST) with supra-therapeutic doses.

7 DRUG INTERACTIONS

- Alcohol: Concomitant chronic alcohol use increases the risk of severe liver damage.
- Warfarin: Chronic high-dose paracetamol use (> 2000 mg/day for several days) may enhance the anticoagulant effect of warfarin and increase bleeding risk.
- Enzyme Inducers (e.g., Carbamazepine, Phenytoin, Rifampin): May accelerate metabolism of paracetamol to toxic metabolite NAPQI, increasing hepatotoxicity risk.

10 OVERDOSAGE